Hazard Analysis
Software Engineering

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Table 1: Revision History

Date	**Developer(s)**	**Change**
Date1	Name(s)	Description of changes
Date2	Name(s)	Description of changes
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1 Introduction

This document covers the hazard analysis for the Ghoti project, a web-based software assistant for speech-language pathologist (SLPs) during sessions. The system captures and analyses a child's speech in real-time, and flags it for substitution errors. Results are presented to the SLP through a dashboard interface and data visualizations. The system also offers in-session exercises to support the SLP in coaching a patient on speech challenges. It is strictly assistive and does not replace the professional judgement of the SLP. This document defines hazard as a property or condition in the system together with a condition in the environment that has the potential to cause harm or loss. The purpose of this document is to identify the hazards of the system, so mitigation strategies and safety requirements can be derived.

2 Scope and Purpose of Hazard Analysis

This hazard analysis addresses the major hazards that could negatively impact the Ghoti project. This includes hazards with speech/audio capture, speech analysis, and interfaces. The analysis will cover the various risks such as with patient data handling, data privacy, accuracy, and performance. The losses that could result from the hazards include violations of PHIPA, privacy breaches, inaccurate analysis of speech and presentation of data, and loss of stakeholder trust. The purpose of this hazard analysis is early identification of any risks that could affect the key components of the system. Identifying these risks will inform the creation of mitigation strategies and proper safety and security requirements. Doing this will minimize risk and potential loss from unaddressed hazards.

3 System Boundaries and Components

[Dividing the system into components will help you brainstorm the hazards. You shouldn't do a full design of the components, just get a feel for the major ones. For projects that involve hardware, the components will typically include each individual piece of hardware. If your software will have a database, or an important library, these are also potential components. —SS]

4 Critical Assumptions

[These assumptions that are made about the software or system. You should minimize the number of assumptions that remove potential hazards. For instance, you could assume a part will never fail, but it is generally better to include this potential failure mode. —SS]

5 Failure Mode and Effect Analysis

Table 2: Failure Mode and Effects Analysis

Failure Mode	**Causes**	**Effects**	**Detection**	**Recommendations**
Phonemic identification model misclassifies patient speech (false positive or negative)	• External noise reduces quality of the audio • Unique vocal patterns or accents of the patient cause incorrect phoneme to be identified • Patient purposely mispro-nounces a sentence (malicious actor) • Technical issues with the microphone • Updates to model could cause regression and diminishing of accuracy for the patient's specific speech sound disorder (SSD)	• Patient's treatment data contains errors, mis-representing their actual progress	• Notify clinician of noisy data • Notify clinician of large gaps in variance • Flag illogical identified phonemes (Phonemes from a language a patient does not know, phonemes with vastly different characteristics than the expected phoneme)	• Allow the patient to try again • Use aggregate data for progress tracking • Display the model's confidence in the phoneme it identified • Display recommendation to limit noise • Allow for microphone test at the beginning of a session and ensure the quality is sufficient to get audio clips from a patient • Allow clinician to test the model by pronouncing the correct phoneme

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Table 2: Failure Mode and Effects Analysis (Continued)

Failure Mode	**Causes**	**Effects**	**Detection**	**Recommendations**
Performance too slow for a live session	• The device where the phonemic analysis occurs is congested or busy • Wi-Fi/connection issues like an unstable connection or congested network can cause high latency • Sessions that use a client-side device may run older or slower specs, causing computation times to be extremely slow	• Valuable time lost in session waiting for a response from the application • Frustration from SLP and patient for extending a session and wasting time	• Measure the time between the start of an audio analysis and its end. Note large time intervals above a given threshold	• For server-side computation methods, scale the number of servers used based on number of customers • For server-side computation methods, if latency is too high, switch to using client-side device for computation • Alert clinician or patient during a session that the connection is slow/unstable
Data breach exposes patient information	• Malicious actors • Data backup to an insecure location	• Software violates PHIPA guidelines • Sensitive medical data from a patient can be taken advantage of	• Monitoring tools for the deep or dark web where sensitive information could be sold • Logs of data access to check whenever the data has been viewed	• Software flags any suspicious transactions in the database • Multiple data backups such that if one data storage location is exposed, it can be isolated and no data will be lost
Race conditions in database access and/or modification	• Multiple clinicians access or edit the database at the same time	• Recently input patient data could be lost	• Timestamps for database viewing or modification are extremely close together	• Suspend the execution of a database action (e.g. view, modify) while a transaction is in progress (transactions include editing database elements, adding database elements, or removing database elements)

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Table 2: Failure Mode and Effects Analysis (Continued)

Failure Mode	**Causes**	**Effects**	**Detection**	**Recommendations**
Authentication or access control failure	• Forgotten username or password Connection cannot be made to authentication server	• Clinician or patient is unable to use the application	• System detects multiple failed logins	• Implement forgotten username or password system • Check connection to authentication server Provide contact information to authentication server support
Data is corrupted or lost	• Data storage hardware is damaged • Software glitch corrupts data • Software attacks that encrypt data (e.g. Ransomware Scheme)	• Patient data and overall progress is lost	• Check connections to all data storage devices • Software tool to check for encrypted or inaccessible data	• Regularly backup data to multiple different data storage devices, if one should be compromised or lost, others will still be available • Check health of data storage devices frequently, should one be experiencing signs of a potential failure, set up plans for replacement
Replacement of a clinician's judgement	• Clinician believes everything the phonemic identification model says at face value • Parents or guardians of patient trust the application and model over clinician	• Model is not always accurate or correct which could be providing false information to the patient. This can cause issues in their development if they believe when they are/are not speaking correctly, the opposite is true.	• Decline in patient's overall performance • Minimal to no corrections given to the model	• Software consistently reminds clinicians and patients that machine learning models are not always perfectly accurate and can be wrong • Check corrections given to the model for inconsistencies and frequency of corrections

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Table 2: Failure Mode and Effects Analysis (Continued)

Failure Mode	**Causes**	**Effects**	**Detection**	**Recommendations**
Patient is frustrated with using the software to do lessons	• Patient struggles to pronounce a phoneme or complete a lesson • Each patient responds differently based on what kind of feedback they receive to improve (e.g. encouragement, straightforward, etc.) • Patient finds the software boring to use	• Patient is unwilling to engage with the software, making treatment using it less effective	• In-session discomfort, annoyance, or anger • Refusal to do lessons • Detect limited improvements to progress	• Ask for patient feedback on improving elements of the application • Allow for choice of feedback type that works well for the patient

6 Safety and Security Requirements

[Newly discovered requirements. These should also be added to the SRS. (A rationale design process how and why to fake it.) —SS]

7 Roadmap

[Which safety requirements will be implemented as part of the capstone timeline? Which requirements will be implemented in the future? —SS]

8 Appendix — Generative AI Use Disclosure

[ChatGPT (version 5) was used to brainstorm ideas for the template for this disclose section. —SS]

8.1 AI Tools Used

[Give the name of the AI tool(s) used and the version. —SS]

8.2 How and Where Used

[Explain what the tool(s) were used for. Examples include: brainstorming, explaining a technical concept, reviewing the document, generating or modifying text, generating or modifying diagrams, identifying errors or omissions or inconsistencies. —SS]

[For each of the uses, identify which parts of the document were affected. —SS]

[You do not need to report every interaction, but you should disclose the uses that materially contributed to your work —SS]

8.3 Verification of AI-Generated Content

[

- Describe how the team checked AI-generated or AI-assisted material before including it in the document.
- In particular, verify factual claims, technical assertions, requirements, calculations, references, and other information for which an AI system may produce incorrect or fabricated results.
- **The team is responsible for the accuracy, completeness, consistency, and appropriateness of the submitted document, regardless of whether material was generated by a human or an AI system.**

—SS]

9 Appendix — Reflection

[Not required for CAS 741 —SS]

The purpose of reflection questions is to give you a chance to assess your own learning and that of your group as a whole, and to find ways to improve in the future. Reflection is an important part of the learning process. Reflection is also an essential component of a successful software development process.

Reflections are most interesting and useful when they're honest, even if the stories they tell are imperfect. You will be marked based on your depth of thought and analysis, and not based on the content of the reflections themselves. Thus, for full marks we encourage you to answer openly and honestly and to avoid simply writing "what you think the evaluator wants to hear."

Please answer the following questions. Some questions can be answered on the team level, but where appropriate, each team member should write their own response:

1. What went well while writing this deliverable?
2. What pain points did you experience during this deliverable, and how did you resolve them?
3. Which of your listed risks had your team thought of before this deliverable, and which did you think of while doing this deliverable? For the latter ones (ones you thought of while doing the Hazard Analysis), how did they come about?
4. Other than the risk of physical harm (some projects may not have any appreciable risks of this form), list at least 2 other types of risk in software products. Why are they important to consider?